

**POLICY AND PROCEDURE FOR THE
MANAGEMENT OF ABUSIVE, DIFFICULT AND
VIOLENT PATIENTS IN GENERAL PRACTICE**

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1 INTRODUCTION

- 1.1 General practitioners (GPs) and their staff will come into contact with patients who may be abusive, difficult or violent. Such behaviour is unacceptable and should not be tolerated. The purpose of this policy document and the guidelines it contains is to set out the PCT's processes for supporting general practice in managing such patients and responding to particular incidents.
- 1.2 Most GPs will at some stage have encountered '**difficult**' patients. These patients are not violent, however their behaviour is still inappropriate and may cause distress and anxiety to GPs, their staff or other patients. Examples include:
 - Abusive and threatening language
 - Aggressive and threatening behaviour
 - Excessive demands made on the services provided by GPs and their staff
 - Generally uncooperative behaviour, e.g. persistent failure to attend appointments
 - Vexatious behaviour, eg a patient who continually makes complaints without justification
- 1.3 The PCT needs to ensure that GP practices have systems in place to help manage those "difficult" patients.
- 1.4 **Violent patients or potentially violent patients** present a more serious risk and the role of the PCT is to ensure that alternative service options are in place to minimise any risk to GPs and their staff.
- 1.5 The Department of Health issued HSC 1999/226 "Campaign to stop violence against staff working in the NHS: NHS zero tolerance zone" and HSC 2000/001 "Tackling violence towards GPs and their staff; The NHS (Choice of Medical Practitioner) Amendment Regulations 1999". This guidance stated that professionals and staff working in the health service would no longer tolerate aggression, violence and threatening behaviour by the public.
- 1.6 The guidance defines violence as: *"Any incident where staff are abused, threatened or assaulted in circumstances related to their work, involving explicit or implicit challenge to their safety, well-being or health"* – European Commission DG-V 1997.

There is no guidance defining a potentially violent individual. However, a person may be defined as such if it is clear that the individual has a history of actual or potential violence in another setting or has demonstrated persistent behaviour in a health care setting which has been categorised and dealt with as minor and medium incidents but has failed to respond to appropriate measures and

boundary setting (see categorisation of risk matrix at **Appendix 1**).

- 1.7 Patients will be designated as violent by the PCT if an incident has occurred requiring police involvement and necessitating immediate removal from a practice's list. The above definition covers violence enacted by patients, their relatives or carers, or indeed anyone posing a threat to GPs, their staff or other patients. The definition recognises that violence may take on many forms, for example:
 - Significant mental or physical trauma
 - Threatening or intimidating behaviour
 - Stalking and harassment
- 1.8 In the event that a practice believes that a patient's **potential risk** of violence is unmanageable but there has not been the need for police involvement, then the practice can refer the patient to the PCT's Tackling Violence Panel for consideration for inclusion on the Tier 2 Scheme. The practice would need to give information on the patient's previous behaviour or ongoing challenging behaviour. The patient should be referred to the Tackling Violence Panel using the Removal Form at **Appendix 11**.
- 1.9 The right of individuals to receive mainstream primary care services remains a fundamental principle of the NHS and violent or potentially violent patients who have been subject to immediate removal from practices' lists will not be excluded from receiving primary care services. However, their behaviour may compromise their right to where they access such services and regulations allow a PCT to override considerations about the distance between their residence and where they receive primary care services.
- 1.10 The PCT has established the Tier 2 Scheme for the provision of general medical services to violent or potentially violent patients.

2 SCOPE OF THE POLICY

- 2.1 The policy covers procedures and processes for supporting GP practices in managing difficult, abusive and violent patients and the arrangements for admitting patients onto the Tier 2 Scheme, following an incident or potential incident of violence.
- 2.2 The policy follows a risk categorisation matrix, set out in **Appendix 1**, classifying risk into minor, medium or major.
- 2.3 The following areas are addressed:
 - The organisational responsibilities for taking decisions on the management of violent or potentially violent patients and violent incidents.

- The processes required by GPs to notify the PCT of a violent or potentially violent incident and also of the removal of a patient from a practice's list.
- The appropriate action to be taken in the event that a violent or potentially violent incident is reported by a GP.
- The processes by which the Primary Care Directorate will link into the wider PCT approach to managing violence in relation to PCT staff and other NHS staff in the wider health community.

2.4 The policy is designed to be followed by members of Worcestershire PCT's Primary Care Directorate and GPs and their staff.

3 POLICY STATEMENT

3.1 ABUSIVE AND DIFFICULT PATIENTS (UP TO AND INCLUDING TIER 1)

- 3.1.1 Patients who access primary care services may be ill, distressed, angry, disturbed or deprived. These factors may lead some patients to exhibit inappropriate behaviour. The nature of the work carried out in primary care means that on occasions GPs and their staff may be subjected to such behaviour. This is unacceptable and should not be tolerated. However, GPs and their staff may be able to use a number of approaches to reduce such occurrences and prevent further incidents in the future.
- 3.1.2 In the event of an incident of abuse or difficult behaviour, a written record should be made of what has occurred. When recording such events it should be borne in mind that patient notes may be forwarded to other practices and may also be requested by the patient themselves. The recorded notes should use standard descriptions where appropriate and avoid emotive language.
- 3.1.3 The incident should be reviewed as part of determining the practice's response. In particular the following should be taken into consideration:
- was the patient's behaviour the result of a medical condition, whether or not capable of being treated. This could take one of many forms, such as mental illness (mania or dementia), the effects of drug and alcohol misuse, or a personality disorder.
 - was the patient's behaviour possibly due to underlying physical causes (eg pain, hunger, sleep disorder) or physiological causes (fear, frustration, low self-esteem, feeling vulnerable or threatened) which may be exacerbated by unemployment, poor housing, family/marital difficulties or language barriers.
 - was the patient's behaviour influenced by environmental factors in the surgery – eg lack of seats, heat/cold, poor toilet facilities, or lack of information. Delays in seeing a healthcare professional may also be a

contributory factor.

- was this incident part of a pattern of persistent or regular inappropriate behaviour or an atypical one-off.

3.1.4 Following an incident of abusive or difficult behaviour the practice may wish to consider the following options prior to taking more permanent action such as removal from the practice list:

- undertaking a conciliation process with the patient. This is not intended to appease difficult patients but to ensure that incidents are dealt with in a fair and balanced way after proper investigation and consideration of the facts (example process in **Appendix 6**).
- writing a formal warning letter stating that such behaviour will not be tolerated and that any future occurrence may result in removal from the practice list (examples at **Appendices 2-5**).

The patient may be given the opportunity to explain their behaviour and apologise for the incident.

- if a practice considers additional support is necessary in managing a perceived risk from a patient, then it may request the support of a security guard service under the Tier 1 scheme, details of which are provided at **Appendix 10**.

3.1.5 GP practices have the right to remove a patient from their list for abusive and difficult behaviour. This may be an entirely appropriate response following either a single incident of sufficient magnitude to require an immediate response or following a pattern of behaviour, where the patient has been provided with a written warning during the past 12 months. If practices wish to remove a patient from the list they are responsible for informing the PCT's Patient Data Team and providing further detail on the incident concerned.

Following a **low-medium risk incident** of abusive or difficult behaviour, the patient will be allowed to re-register with another practice of their choosing. In these circumstances the practice should write to the patient and indicate that they will be removed from the practice's list in seven days and that they should seek an alternative practice. Once registration has been completed, then details of the incident will be provided to the patient's new practice as part of the transfer of patient notes. Allocation would only occur if a patient was unable to find an alternative practice.

Where a patient is unable to register with another practice the PCT will allocate to a practice in line with normal procedures. The Patient Data Team will make the practice aware of the incident leading to removal at the time of allocation in order to enable the practice to consider using a pre-registration interview to set down

future expected standards of behaviour.

3.2 VIOLENT OR POTENTIALLY VIOLENT PATIENTS (TIER 2)

3.2.1 As stated in the introduction, a patient will be classified as violent following an incident involving the police and requiring immediate removal from a practice's list. A patient may be classified as potentially violent if:

- (a) there is clear evidence that the individual has a history of actual or potential violence or
- (b) where there have been a number of previous incidents involving the patient which have been previously categorised and dealt with as minor and medium incidents (see **Appendix 1**).

Where there is evidence that the patient has repeatedly failed to comply with appropriate measures to manage their behaviour and the risk of potential violence remains then the PCT's Tackling Violence Panel will place the patient on the Tier 2 Scheme.

3.2.2 The PCT should be immediately notified of any incidence of violence or threat of violence which occurs on practice premises or on a home visit using the Removal Form at **Appendix 11**.

3.2.3 GPs and their staff should be encouraged to work fully with the police in the event of an act of physical violence and for charges to be pressed.

3.2.4 An act of violence should be treated as a serious untoward incident and the event fully recorded in order to enable any lessons from the incident to be learned. The practice should complete an Incident Report Form which will assist the PCT in assessing the wider and future risk associated with this patient.

3.2.5 The PCT should also be informed of any violent incidents which occur in other local healthcare provider settings, eg OOH Services, Walk in Centre. The Primary Care Directorate should also receive notification from the PCT's Provider Services, Mental Health and Acute Trusts, when a patient has been given a written warning or placed under a procedure of care (treatment withheld) by PCT or Trust staff. The PCT's Primary Care Directorate will contact GPs to inform them that one of their patients has been designated violent in another healthcare setting and in discussion with the practice a judgment will be made as to whether there is sufficient risk in primary care for the patient to be placed on the Tier 2 Scheme.

3.2.6 In addition, the PCT can allocate a patient to the Tier 2 Scheme in the following circumstances:

- Patient moving into the area who is currently on the equivalent of the Tier 2 Scheme elsewhere.

- On release from prison, if the PCT is alerted by the Probation Service that the individual poses a significant risk.
- Patients who have previously been placed on the scheme and who, on return to mainstream primary care services, threaten further disruption.

3.2.7 The PCT's Tackling Violence Panel will be convened to make an assessment of the current and future risk to GPs, their staff and other patients following an incident of violence or potential violence. The panel will comprise:

- Deputy Head of Primary Care
- Primary Care Contracts Manager
- Representative of the Tier 2 Scheme Provider
- LMC Representative

Where it is not possible to convene a Tackling Violence Panel due to time constraints, any one of the following can make a decision to allocate a patient to the Tier 2 Scheme for a probationary period of 2 weeks in consultation with a representative of the Tier 2 Scheme provider:

- Director of Primary Care
- Head of Primary Care
- Deputy Head of Primary Care

Ratification of that decision by the PCT's Tackling Violence Panel should then be sought.

3.2.8 The purpose of the Tackling Violence Panel will be to confirm that the patient meets the definition of a violent patient or potentially violent patient, agree the patient's admission to the Tier 2 Scheme and assess their likely risk to GPs, their staff and other health professionals. The assessment of risk should take into account the following criteria and should include a medical opinion where deemed necessary by the Panel.

- Description of incident – physical violence / threatening behaviour / verbal abuse
- Opinion of practice where incident occurred
- Previous history
- Mitigating circumstances
- Clinical opinion
- History of substance misuse/mental illness
- Family circumstances
- Whether charges were pressed
- Likelihood of re-occurrence of violence

- Severity of threat
- Previously been on another PCT's violent patients scheme

3.2.9 The Panel also needs to agree an appropriate information cascade based on assessment of risk (eg to cascade within locality or countywide). The agencies/providers below will be informed:

- GP Practices
- OOH Provider
- Walk-in Centre
- Local Acute Providers

Where applicable, other agencies/providers may be informed of the patient's registration with the Violent Patients Scheme eg:

- PCT's Provider Services
- Drug & Alcohol Service Providers
- Mental Health Service Providers
- Social Services
- Voluntary Services
- Neighbouring PCT where appropriate

The panel will agree a review date for all patients included on the Tier 2 Scheme. The panel will also ensure that the general operation of the scheme is satisfactory.

3.2.10 Notification of allocation to the Tier 2 Scheme will be sent to those independent contractors and health professionals as agreed by the Tackling Violence Panel. Should the patient present at the premises of any of the service providers who have been notified of his/her allocation he should be dealt with in a courteous manner but the Primary Care Directorate should be notified immediately.

3.2.11 Worcestershire LMC will be informed of all incidents of violence and their classification by the PCT's Tackling Violence Panel.

3.2.12 Those contractors looking after the patients on the Tier 2 Scheme will be required to complete a patient behaviour report following each consultation with the patient, which will be made available upon request to the PCT's Tackling Violence Panel for review purposes (example shown at **Appendix 7**).

3.2.13 The PCT's Tackling Violence Panel should review on a 6 monthly basis those patients on the Tier 2 Scheme and determine at what stage it would be appropriate for them to be returned to mainstream primary care.

3.3 RIGHT OF APPEAL

All patients have a right of appeal against the sanctions imposed by the PCT.

Any appeal should be made in writing to the PCT's Chief Executive. Appeals will be processed and managed in accordance with the PCT's Complaints Policy.

3.4 ZERO TOLERANCE CAMPAIGN AND SUPPORT FOR GPs

3.4.1 The PCT is committed to zero tolerance of violence in all primary care settings. All healthcare workers have the right to work without fear of abuse or assault and patients are expected to cooperate with this approach.

3.4.2 The PCT has produced good practice guidance to support GPs in tackling violence initiatives (**Appendix 8**).

4 MONITORING AND AUDITING

4.1 There are a number of aspects of this policy which will need to be monitored and audited on a regular basis. The overall responsibility for this will lie with the PCT's Primary Care Directorate. Key tasks in relation to monitoring and auditing the policy will be undertaken by specific individuals or parts of the organisation.

4.2 The Patient Data Team will produce a quarterly report listing all patients removed from a GP practice due to abusive, difficult or violent behaviour. The report will classify incidents as Minor, Medium or Major.

4.3 The progress of patients on the Tier 2 Scheme will be assessed annually through a meeting of the Tackling Violence Panel. Any issues relating to the overall operation of the scheme will be discussed through the Panel.

Categorisation of risk and required action following incidents of abusive, difficult or violent behaviour

ABUSIVE AND DIFFICULT PATIENTS

Category of incident	Type of incident	Action by practice	PCT action
Minor	Abusive language Atypical behaviour	- Practice maintains a record of the incident - Analysis of incident to determine underlying cause - Practice should send out a warning letter to the patient explaining that if there is a repetition of their behaviour they will be removed from the practice list - Practice should consider conciliation thus providing an opportunity for an apology to be made	None required
Medium	Persistent abusive language and inappropriate behaviour or a more extreme incident of abusive language or difficult behaviour	- Practice makes a decision that this is a “medium” incident - Practice maintains a record of the incident and informs the PCT’s Primary Care Directorate - Analysis of incident to determine any underlying cause - Further warning letter - Conciliation if not used before	Consideration given to advising the practice to involve the police of the incident if judged to be more serious.
		Practice decides that the patient may be retained on the practice list but that some additional support or action may be required, eg final warning letter, conciliation interview with a PCT mediator present, a security presence in surgery (Tier 1)	Possible source of support to the practice is discussed.
		Practice informs the PCT’s Patient Data Team that it is writing to the patient to explain that because of their behaviour they will be removed from the practice list in 7 days and that they should find an	Patient will be allocated to a practice by the Patient Data Team in

		alternative practice. The letter should also give contact details for the Patient Data Team in the event that they have difficulty registering elsewhere.	the event that they have difficulty finding a practice. • Patient Data Team advises the practice receiving the allocation of the patient's behavioural history. • Patient Data Team will maintain a file note to inform the patient's new practice of their previous behaviour.
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VIOLENT OR POTENTIALLY VIOLENT PATIENTS

Category of incident	Type of incident	Action by practice	PCT action
Major	An incident of violent or threatening behaviour where the police have been called and which necessitates immediate removal from the practice list.	• Police are called to the incident and the practice follows their advice on how to proceed in terms of possible criminal proceedings. • Practice immediately provides the PCT with details of the incident, the patient's details and the police incident number. • Practice request the PCT to remove the patient from their list with immediate effect (using patient removal form at Appendix 11 faxed to the Patient Data team).	• Immediate assessment of risk by the Primary Care Directorate to determine required action including informing other practices and organisations. • Panel agreement that the patient should be admitted to the Tier 2 scheme. • Other organisations formally notified of the patient's admission to the scheme including other practices, Provider Services, the OOH Service and the Acute Trust if appropriate. • Patient notified in writing of their removal from the practice's list and of their admission to the Tier 2 scheme.

	Patient has a history of actual or potential violence or persistent threatening behaviour which has previously been categorised and dealt with as minor and medium incidents within the last 12 months. Patients moving into area who were on previous PCT's equivalent of Tier 2.	Practice makes a written request to the Deputy Head of Primary Care for the patient to be admitted to the Tier 2 scheme.	Panel assessment of current and future risk, confirmation whether or not the patient meets the definition of being a violent or potentially violent patient and a decision whether or not to admit the patient to the Tier 2 scheme.
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Example Letter to Patient Following Alleged Inappropriate Behaviour

Recipient's Name
Address Line 1
Address Line 2
TOWN
Postcode

Dear [Mr/Mrs/Ms XXXXX]

Re: Title of Letter

I am writing to you about an (*alleged*) incident / situation* on (date) at (place) where (*I understand that*) the following occurred (describe incident). If you disagree with this description of what occurred, or there are circumstances that we are unaware of, which you believe would help to explain or excuse what happened, please let us know, preferably in writing, as soon as possible. If you would like to discuss this matter, please contact us and we will arrange a meeting at a time that is mutually convenient to discuss these concerns. If you wish, you can be accompanied by a friend or Worcestershire Primary Care Trust (with whom we have an NHS service contract) can provide an independent person to act as a facilitator at the meeting.

In this practice we have a policy for dealing with situations in which our services are abused. Where there is serious or persistent misbehaviour it can result in a patient being removed from our practice list. Before taking any action we would like to hear from you.

Please remember that we are here to help you. All that we ask is that our patients act reasonably and are considerate of the demands they make of us.

Yours sincerely

Name
Job Title

Example Letter to Patient Following Alleged Repeat of Inappropriate Behaviour

Recipient's Name
Address Line 1
Address Line 2
TOWN
Postcode

Dear [Mr/Mrs/Ms XXXXX]

Re: Title of Letter

I am writing to you about an (*alleged*) incident/situation on (date) at (time) at (place) when (*I understand that*) the following occurred (describe incident). You will recall that we wrote to you on (date), copy letter enclosed for information, about an incident/situation in which (describe incident). We warned you on that occasion of our policy of seeking removal from our list of patients who seriously or persistently abuse our services.

Before taking any action we would like to hear from you if there are circumstances relating to this latest incident/situation that we are unaware of, which you believe would help to explain or excuse what happened. If you would like to discuss the matter please contact us and we will arrange a meeting at a time that is mutually convenient.

If we do not hear from you by (date) we may decide to request that Worcestershire Primary Care Trust remove you from our list.

Yours sincerely

Name
Job Title

Enclosure (if appropriate)

Example Letter Following Investigation of Alleged Misbehaviour

Recipient's Name

Address Line 1

Address Line 2

TOWN

Postcode

Dear [Mr/Mrs/Ms XXXXX]

Re: Title of Letter

Further to my letter of (date), copy enclosed for information, I am writing to you about the incident/situation* on (date) at (time) at (place) when (describe incident).

Having investigated the matter (and in view of your comments) we have decided to take no action/to warn you that should any repetition of this sort of behaviour/incident or anything similar occur in the future, we may request Worcestershire Primary Care Trust remove you from our practice list.

We hope that we will enjoy an improved relationship with you in future.

Yours sincerely

Name

Job Title

Enclosure (if appropriate)

Example Letter When Removal From a GP List is Preferred Option

Recipient's Name
Address Line 1
Address Line 2
TOWN
Postcode

Dear [Mr/Mrs/Ms XXXXX]

Re: Title of Letter

Further to my letter of (date), copy enclosed for information, I am writing to you about the incident/situation* on (date) at (time) and (place) when (describe incident).

Having investigated the matter we have decided to request that Worcestershire PCT remove you from our list. This will take effect in seven days time and in the meantime you should register with another practice.

In the event that you have difficulty in finding another practice then you should telephone the PCT on 01905 760000 and ask for the Patient Data Team.

Yours sincerely

Name
Job Title

Enclosure. (if appropriate)

Example of Independent Contractor Conciliation Process

Stage 1

- Find out what has happened and try to identify the cause
- Agree the process, eg:
 - try to speak to the patient (informally/privately) at the time of the incident or
 - invite them to the practice for discussion or
 - write to them with the offer of a meeting to discuss the incident/behaviour (it may be useful to use a specially skilled or sympathetic member of the practice to facilitate this)
- During the meeting find out the reasons for behaviour/incident. Discuss the consequences of repetition etc. Keep a file note of discussions and use incident report form
- Consider inviting PCT staff to advise/conciliate and/or contact other agencies if required

Stage 2

Where repetition of incident/behaviour occurs or improvement required has not taken place:

- Send further warning letter (inviting comments as before)
- Notify PCT and consider outside intervention if not used before

Stage 3

If there is a further reoccurrence of behaviour complained of:

- Send final warning letter
- Notify PCT

Stage 4

If there is a further reoccurrence:

- Consider requesting the patient to be removed from the practice list giving reasons
- Notify patient of removal giving reasons as appropriate, with reference to previous warnings

STRICTLY CONFIDENTIAL

Patient Behaviour Report

To be used for patients who have been allocated to Worcestershire PCT's Tier 2 Scheme

To be completed by the attending General Practitioner following all consultations with the patient

EITHER TO BE RETAINED ON THE PATIENT'S NOTES OR FORWARDED TO THE PCT'S DIRECTOR OF PRIMARY CARE

Name of patient

Address of patient

.....

Date of Birth

Name of attending practitioner

Date of consultation

Questions in relation to behaviour:

At any time during the consultation did this patient express any violent or threatening behaviour towards yourself or your colleagues?

YES/NO

In your opinion do you believe that this patient may be a continuing threat to those treating him/her or their family, or to those in the immediate vicinity?

YES/NO/POTENTIALLY

Any other relevant information

.....

.....

.....

.....

Signature of attending practitioner..... Date

A GOOD PRACTICE GUIDE

Violence and Aggression towards General Practitioners and their Staff

Background

This guide has been produced by Worcestershire PCT's Directorate of Primary Care and Worcestershire LMC. The guide also takes into account the Health Service Circulars HSC 1999/226 "Campaign to stop violence against staff working in the NHS: NHS zero tolerance zone" and HSC 2000/001 "Tackling violence towards GPs and their staff; The NHS (Choice of Medical Practitioner) Amendment Regulations 1999". This guide relates to all aspects of practice business where violence could potentially be a problem.

Definition of Violence

The definition of work related violence is not subjective. Violence means:

"any incident where staff are abused, threatened or assaulted in circumstances related to their work, involving explicit or implicit challenge to their safety, well-being or health" – European Commission DG-V 1997.

Legal Framework

The legal responsibilities for employers to protect their staff are framed by both National and European health and safety legislation and by their common law duty of care.

Healthcare is integral to the communities within which it is delivered. Consequently, fostering good relations within the community and with organisations such as the police are important in reducing violence against staff in the NHS. The Crime and Disorder Act 1998 requires local authorities and police to co-operate with other bodies including NHS trusts to formulate and implement crime and disorder strategies. **Appendix A** offers additional information.

Your Role

The aim of this guidance is to establish YOUR security role. Whether you are a doctor, nurse, receptionist, caretaker or cleaner, you have to be SECURITY CONSCIOUS. As a member of staff it is your duty to keep your eyes open, look for unlocked doors and windows, challenge strangers, carry out security procedures thoroughly and report all incidents. OVERALL THAT MEANS WORKING TOGETHER. YOU MUST NOT BE AN EASY TARGET!

Assessing the Risk of Violence

Independent Primary Care Contractors should develop local prevention strategies. These should take account of the *environment* in which you are delivering the service, involve *training* of staff and an assessment of *communication* processes within your practice.

Environment

The environment in which staff work, patients are treated and other members of the

public visit can have a significant influence on behaviour. You should consider assessing environmental factors such as cleanliness, light, temperature, the adequacy of space, control of access and signage, as well as the provision of e.g. private room(s). This will ensure that the physicality of a place doesn't trigger or exacerbate a stressful situation. **Appendix B** offers some useful tips. The use of CCTV has also been well documented as a good disincentive to anti-social and/or unlawful behaviour. West Mercia Police's Architectural Liaison Officers are trained to offer advice on environmental factors, their services are free and their details are included at **Appendix A**.

Reception areas are often the first part of the practice that patients see. Naturally, many of the people who walk into a practice are either under a great deal of stress, in pain or very upset. The reactions of these people can therefore never be predicted. It is vital that we do everything possible to keep people occupied whilst they are waiting in reception. We must help them to stay calm. Here are some pointers to help reduce the threat of violence and aggression in reception areas:

Training

Staff working in the NHS should know that their safety comes first. They should not be in situations that make them feel unsafe. However, if they are, they need to know how to deal with them. Appropriate staff training is therefore crucial. You should assess the risks to staff members and analyse their training needs. In doing so it will be possible to gauge the sort of training they require according to their roles and ensure that this is appropriate to the degree of risk an individual employee faces.

Training should be up-to-date, relevant, and purposeful, backed by evidence, given by experts and include scope for feedback. Ideally, independent contractors and their staff should receive the same training to ensure continuity. **Appendix C** offers useful contact points for training provision.

Communications

Maintaining effective communication channels with staff and particularly those working in the community (e.g. home visit) is important. The communication needs of staff and the measures that can be taken to minimise any risk to their safety need to be considered. These might include:

- Providing panic-button alarms and appropriate two-way communications systems such as mobile phones, which are increasingly being used by staff whose work places them in vulnerable situations. **Appendix E** gives some useful tips.
- Establishing protocols for informing members of staff that a colleague is out, where they have gone and their approximate return time. Procedures for reacting to protocols should also be in place.
- Improving communications between all members of the Primary Care Team, keeping others abreast of potentially problem patients.

Avoiding Aggression

Avoiding aggression involves a three-point plan; *prevention*, *controlling* the situation and *de-escalating* the situation. These are discussed in more detail below:

Prevention – there are several danger signals, which could include behaviour of others, your own behaviour, workplace or situational indicators. Staff should remain vigilant, think defensively and respond immediately to danger signals. **Appendix D** offers some useful tips on personal safety.

Control – it is important to ensure you stay in control of a situation. Make sure you breathe deeply, maintain a barrier, stand side on, use open hand gestures and maintain eye contact.

De-escalation – this can vary with the particular situation or individual concerned but strategies could include acknowledging the aggressor's mood, offering help, asking open ended questions, trying to resolve the problem, withdrawing or summoning assistance.

What to do if Prevention Fails

Independent Primary Care Contractors are encouraged to work fully with the police in the event of an act of physical violence and for charges to be pressed. The PCT should be notified immediately of any incident of violence or threat of violence which occurs on practice premises or on a home visit. Independent Primary Care Contractors are encouraged to treat any incidents of violence and aggression as a serious untoward incident and report it to the PCT.

What about Victims?

If there is a violent incident involving your workforce you will need to respond quickly to avoid any long-term distress to employees. It is essential to plan how you are going to provide them with support, before any incident occurs. Depending on the extent of the incident you may wish to consider the following:

Debriefing – victims (and witnesses) will need to talk through the experience as soon as possible after the event. Remember that verbal abuse can be just as upsetting as a physical attack.

Time off work – individuals will react differently and may need differing amounts of time to recover. In some circumstances they might need specialist counselling (can be arranged through Occupational Health).

Legal help – in serious cases legal help may be appropriate.

Other employees – may need guidance and/or training to help them to react appropriately.

Next Steps

Hopefully, this guidance will have made you more aware of your responsibility to

security within your workplace. Effective security is all about good housekeeping and management. Sometimes a cultural change is required, **policies must involve clinical and support staff**. No one is exempt.

Crime prevention strategies can be budgeted for over a period of time. Good crime prevention will improve quality of life and bring economic returns. Security issues should play a part in all meetings and training courses. Independent Primary Care Contractors have a duty to care for patients and staff. Staff have a duty to care for patients and visitors. If sensible working practices are adapted across the board, everyone will be a winner. There may be instances where patients show early signs of inappropriate behaviour and it may be appropriate to consider an Acceptable Behaviour Contract, details of which are provided at **Appendix F**.

The following security checklist provides a quick summary of the information contained within this guidance:

- Enforce all security regulations in your practice
- Know all security responsibilities and procedures
- Inform others of security procedures
- Stay alert and assertive
- Avoid security short cuts
- Always keep valuables out of sight
- Mark all property with the practice postcode
- Know what to do in an emergency
- Always act within legal guidelines
- Report suspicious incidents immediately
- Use your security facilities to the full
- Ask your local police Architectural Liaison Officer for advice

In Short

Don't be an easy victim! You will probably be familiar with many security solutions in your workplace. Contribute your ideas freely and have a positive attitude to practice security.

Suspicious or Criminal Behaviour

CAUTION!

The public are advised never to involve themselves in any incident which puts themselves or others at risk. If you see a crime being committed call 999.

Ask your local Architectural Liaison Officer about specialist training and advice.

The law on common arrest is as follows:

It is not necessary to touch or lay hands on a person to arrest them. You must simply inform the person immediately that he or she is under arrest and explain the grounds for the arrest. Even if the person has escaped before all of the information has been given, the arrest will still be lawful.

Although it isn't vital that all of your words are the exact ones laid down, the following statement will be effective:

"I am arresting you for You do not have to say anything unless you wish to do so, but what you say may be given in evidence".

You can arrest someone:

- Who has committed, or you reasonably suspect has committed, an assault where injury has occurred, which requires stitches for example, but not a slight bruise or a graze.
- Who damages property (where the damage is not accidental) or you reasonably suspect has damaged property.
- Who steals property where they have no right to take the property, or you reasonably suspect they have taken such property.

Trespassing on land or property is a civil wrong, NOT A CRIME. Trespassers can be sued for damage and an injunction sought. They cannot be arrested solely for trespass.

Architectural Liaison Officer (ALO) Contact Details

ALO North Worcestershire

Kidderminster:

Mr Peter Aston

Tel: 0300 333 3000 **Ext:** 3907

Email: peter.aston@westmercia.pnn.police.uk

Bromsgrove:

PC Stan Baker

Tel: 0300 333 3000 **Ext:** 3217

Email: stan.baker@westmercia.pnn.police.uk

ALO South Worcestershire

PC Mike Stephenson

Tel: 0300 333 3000 **Ext:** 4978

Email: mike.stephenson@westmercia.pnn.police.uk

Protocol for Premises Security

This protocol is intended to provide practical suggestions on how you can work to improve the safety and security of the physical environment from which you operate. It covers areas of best practice that can help to reduce the likelihood of aggressive behaviour and lists the minimum specification of security measures in terms of equipment that should be placed to protect staff, patients and visitors to the practice.

- Reception/Waiting areas should be kept clean and hospitable. Up to date reading material or the Life Channel could be provided as a form of entertainment to reduce boredom, frustration and anxiety.
- Space should be properly planned to avoid overcrowding and facilitate the movement of people.
- Temperature should be controlled during seasonal extremes to maintain an environment that is comfortable.
- Comfortable and appropriate seating should be provided i.e. suitable for people with disabilities. Seating should be secured to the floor if the layout and use of the area allows.
- Signs should be clear, simple and visible and direct people to the appropriate location e.g. treatment rooms, toilets.
- Where artificial lighting is used, ensure that this is adequate and maintained to avoid flickering and failure.
- Car parks and external concealed areas should be adequately lit using either automatic or activated lighting.
- Access to the premises should be controlled and monitored. Consider the use of a single entrance/exit where possible.
- Use high quality secure lockable doors where patient/visitor access is restricted.
- Implement a routine for locking/unlocking the premises at the beginning and the end of the day. Two people should be present where possible.
- Reception desks should be wide enough to prevent potential aggressive patients reaching over.
- More than one receptionist should be available at the front desk where possible to allow a patient to be taken to one side if felt necessary, or so that a senior member of staff can be summoned without leaving reception unmanned.

- Keep patients informed of any delays to reduce frustration.
- Panic Alarms should be fitted in the reception area and all consulting rooms, out of the reach of children but easily accessible. Regular testing of the panic buttons and the planned response time is essential.
- A dedicated phone line or posting messages on the computer should be used to communicate quickly and discreetly between the reception area and consulting rooms.
- Independent Contractors/Practice Staff chairs in consulting/treatment rooms should be sited closest to the door for easy escape from a potentially aggressive situation.
- Mobile phones should be provided for staff working outside the practice environment and a “call in” system implemented.
- Installation of CCTV should be considered obtaining specialist advice from a reputable security company.
- Develop relations with your local police and crime prevention officer to provide ongoing support and advice.

West Mercia Police’s Architectural Liaison Officers offer their advice free of charge.

Training Providers

NHS Counterfraud and Security Management Service (CFSMS) are able to deliver course in conflict resolution and lone working.

Contact Details:

Conflict Resolution Project Team

Tel No: 02476 245514

or email: conflictresolution@cfsms.nhs.uk

Non-Abusive Psychological & Physical Intervention (NAPPI) UK Ltd is able to deliver a variety of courses.

Contact Details:

NAPPI UK Ltd, 64 Falsgrave Road, Scarborough, YO12 5AX

Tel No: 01723 353353

Suzy Lamplugh Trust delivers a wide range of personal safety courses

Contact Details:

Suzy Lamplugh Trust, National Centre for Personal Safety, 218 Strand, London WC2R 1AT

Tel No: 020 7091 0014

Be Prepared! Some useful hints on personal safety

Staying safe is very much a matter of common sense. The unfortunate thing is however, that when we feel threatened or vulnerable, common sense is something we tend to forget. Naturally, our safety depends very much on the actions of others, but by following some simple guidelines we can reduce the risk of attack and help to ensure our own safety and well being.

Many Independent Primary Care Contractors and their staff work unpredictable hours. This often entails travelling home later at night or early in the morning when public transport is not widely available. The following guidelines can apply to anyone in any situation. Think how much more relevant they are to you and when your working hours make you so much more vulnerable.

Walking

- Keep to main, well-lit paths and don't take lifts from strangers. Carry a personal alarm. Walk facing the traffic.
- Think about your clothing, wear something sensible that doesn't attract attention and don't wear shoes that may make it difficult to escape a difficult situation.
- If attacked shout and scream, make as much noise as possible.
- Be aware of your surroundings especially at cash points, telephone booths, near pubs and clubs.
- Tell someone where you are going, which route you are taking and when you expect to be back.
- Don't carry large amounts of money and hide valuables from view.

In the Car

- Keep your vehicle in good mechanical order and make sure that you always have sufficient petrol.
- Know your route and always have a map handy. Park in a well-lit, easily visible area.
- Ask someone to walk you to your car.
- Lock all your doors when driving along and keep personal belongings out of sight, NOT on the car seat next to you.
- If you breakdown at night think before you act:
 - Are you safe inside the car?
 - Is the car in a dangerous position?
 - Are you in a well-lit residential area where you can call for help?
 - Is it likely that a police patrol vehicle will be close by?
 - Choose the option which involves least risk and report any suspicious behaviour or incidents immediately.

In the Surgery

There should be a system ready so that we know how to deal with attacks if they

happen. If you are new to a surgery (e.g. locum) ask what the system is:

- Develop a way of calling for help: an alarm code, bell push etc
- Call for help (and the police, if necessary) in plenty of time
- Write a full report on the incident (see **Appendix G**)

Violence against Staff

In the NHS violence against staff is an outward sign of frustration felt by many patients in our care. Fortunately, most people are able to control their emotions, but Independent Primary Care Contractors and their staff are more likely than most to come across those who cannot. Here are some guidelines concerning violent patients and how to cope with them:

- Look for signs of aggression such as uneven speech, shaking, changes in complexion and breathing.
- Stay cool, think as a healthcare professional.
- Immediately assess any dangerous situation. Think:
 - What has happened?
 - what might happen?
 - Is the person a danger to others?
 - Is the person a danger to themselves?
 - What was the cause of the situation?
- If you can deal with the situation yourself do so, if not, get help by pushing an emergency button, calling for help, getting someone to take a message to a colleague.
- Protect other patients and yourself.
- Remember, damaged equipment is better than injured people.
- Be positive, tactful, serious and assertive.
- Don't be confrontational or patronising.
- Try to reduce the tension and remove the cause of aggression.

Threats to staff on home visits

No matter how good security is in the practice, there will be times when an independent contractor has to go out and visit patients in their own homes. Naturally, it is vital that we take all possible precautions to ensure that independent contractors are safe all of the time. Here are some practical tips that will help to reduce the risks faced by independent contractors on home visits:

- Leave details of your itinerary at the practice and advise colleagues of any changes.
- Carry a radio link or mobile phone so that you can advise colleagues of your whereabouts at regular intervals.
- Arrange a code word/phrase that will alert your colleagues that you are in danger.
- Lock all equipment in the boot of your car.
- Keep comprehensive records of all home visits detailing any particular risks. By keeping an up-to-date centralized record system, you will be aware of any potential problems before you visit the patient. Record details such as:
 - Location of property, is it remote?
 - Means of access to property.
 - Attitude of patient.
 - Any dangerous pets or obstacles?
 - Procedural problems, does patient find it difficult to move?

These records will act as an aide-memoire for next time you visit the property and an invaluable source of information for anyone else visiting the property for the first time.

Once you have assessed the risk in visiting a client, you can decide whether or not you need to be accompanied on your visit.

Anticipating an Attack in the Community

Consider the possibility of attack in advance. Ask yourself if you are physically strong enough to defend yourself immediately, or will you play along with the attacker and choose your moment to escape. Whatever you decide, here are some tips to help:

- Move quickly.
- Make as much noise as possible.
- Use your umbrella, hairspray or keys against the attacker. Breathe deeply and steadily.
- Don't argue with the attacker.
- Stay as calm as possible.
- Don't fight with the attacker but do try to escape.
- When you escape head for a well-populated area. One way of boosting self-confidence and protecting yourself is the art of self defence. Naturally the best way to learn self defence is through a regular class but, here are some self defence tips:
 - Self defence is as much about timing as force. You don't have to be the strongest person in the world, but you do have to be able to target your energies effectively. No matter how frightened you are, it is vital that you choose your moment to escape carefully.
 - If you are grabbed from the front, bring your hands up through the attacker's arms. Use the heel of your hand to hit their chin or nose.
 - A poke in the eye will knock the attacker off balance and give you time to escape. Biting the attacker's hand or arm will also have similar effect.
 - If you are grabbed around the neck, take hold of the attacker's little fingers and pull back hard.
 - If you see the attacker coming towards you, scream at them. If they manage to take hold of you, knee them in the groin.
 - If you are grabbed from behind, stamp on the attacker's foot or scrape your heel down their shin.

Remember at all times only REASONABLE force should be used to prevent an attack

Acceptable Behaviour Contracts

Some patients who threaten or proceed to actual violence against Independent Primary Care Contractors and their staff show early signs of inappropriate behaviour before a violent incident occurs. In such cases, it may be appropriate to consider asking the patient to sign an Acceptable Behaviour Contract that clearly sets out what is acceptable and what is unacceptable behaviour when accessing primary care services.

Acceptable Behaviour Contracts have been used with some success by a number of local authorities across the UK, where they can act as a warning to potential problem patients that certain behaviour will result in action being taken to remove the patient from the practice list.

An Acceptable Behaviour Contract should be considered:

- Where a patient shows early signs of inappropriate attitude or behaviour
- As an alternative to removal of the patient from the practice list, in the case of a less serious incident
- When a patient is discharged from a PCT's Tier 2 Scheme to return to mainstream NHS primary care services.

Acceptable Behaviour Contract

Patient's Name

Address

NHS Number

Responsibility and Rights – A Patient Undertaking

Your Rights	Your Responsibilities
<i>(insert name of independent contractor)</i> and their staff owe to me, as a patient, a duty of care and aim to provide services to meet my needs for healthcare and treatment at all times.	I will not behave in any way, which can be considered to be violent or abusive
<i>(insert name of independent contractor)</i> and their staff aim to provide health services that are sympathetic and responsive to my individual needs within the resources which the PCT/Primary Care Independent Contractor has available.	Violence includes any incident where <i>(insert name of independent contractor)</i> and their staff, fellow patients and their carers are abused, threatened or assaulted in circumstances related to their work. An act of violence may involve an explicit challenge to the safety, well being or health of any member of PCT staff, Primary Care Independent Contractor, their staff or other patients. Violent behaviour may include verbal abuse, racial or sexual harassment, threats of injury, abuse of alcohol or drugs, destruction of NHS property, as well as physical acts of violence.
I will treat <i>(insert name of independent contractor)</i> and their staff, fellow patients and their carers and visitors politely and with respect at all times.	<i>(insert name of independent contractor)</i> and their staff are expected to treat me with courtesy and respect.
I will not consume alcohol or take any form of non prescribed medication or drugs whilst on NHS premises	<i>(insert name of independent contractor)</i> and their staff want to deliver appropriate and effective healthcare and treatment to me.
<i>(insert name of independent contractor)</i> and their staff will only restrict or withdraw my rights to care in exceptional circumstances when I have failed to comply with any of my responsibilities in a manner which is deemed unacceptable.	I accept and understand that <i>(insert name of independent contractor)</i> is obliged to provide a safe and secure environment for all its staff and to care for their health and safety. I accept and understand that no member of the <i>(insert name of independent contractor)</i> 's team has to jeopardise their safety in providing me with care.

I confirm that I understand that if my behaviour has been unacceptable and if I do not comply with my responsibilities as a patient, then this can result in the withdrawal of my rights as a patient and I can lose my right to receive mainstream NHS Primary Care Services.

Signature of patient

.....

Print Name (Block Capitals)

.....

Date

Signature of Responsible Independent Primary Care Contractor

.....

Print Name (Block Capitals)

.....

Date

If there is an incident of violence

You should report the incident to the police immediately. It could mean the difference between arrest and the attacker striking again. Don't be afraid to report the incident. Your case will be treated sympathetically and in confidence. In serious cases, police forces have access to specially trained staff who will be able to give you the advice and counselling you need. Ask the operator for an incident number and make a note of it.

Whilst you are waiting to speak to the police:

- Don't drink any alcohol
- Don't wash
- Don't change your clothes
- Write down all the facts that you can remember about the attack, including:
 - How you felt
 - Details of any witnesses
 - Details of any injuries
 - Details of any weapons used
 - Details of any property damaged or stolen
 - Description of person involved (sex, height, age, build, clothing, facial features, ethnicity, hair colour, name if known)
 - Description of vehicle (make, colour, registration, number of occupants)

When the police attend, ask officer(s) attending for their collar number(s).



TIER 1 SCHEME – RETAINING POTENTIALLY VIOLENT PATIENTS WITHIN GENERAL PRACTICE – SUPPORT AND SECURITY ARRANGEMENTS

1 Introduction

This procedure is a result of Worcestershire Primary Care Trust's commitment to zero tolerance and supports Independent Primary Care Contractors and their staff in tackling the violence issue.

The scheme will ensure that practitioners and their staff are covered by appropriate security arrangements in potentially threatening circumstances. Whilst the PCT supports a zero tolerance to verbal abuse, this procedure is aimed at dealing with potential physical violence.

2 Persons covered by this policy

This policy covers all GPs and their staff whose responsible list sits with the PCT. The policy should be interpreted to cover all potential acts of violence in primary care, including home visits. It should be noted that the policy does not replace the use of 999 following ad hoc instances of violence. It is intended that this policy will support the existing procedures for immediate acts of violence and will provide security for independent contractors and their staff during planned consultations with potentially violent patients.

3 The security arrangements

The PCT has a cost per case contract with a local security firm to provide security arrangements on request for GPs and their staff covered by this policy. Security is proposed as follows:

- (a) **Ad hoc acts of violence** – dial 999
- (b) **Home visits** – the local security firm will accompany a GP on any domiciliary visit where the risk assessment identifies there is a possibility of practitioners being subject to physical violence. The security guard will meet the independent contractor at an agreed time and place before the visit begins. If the visit is an emergency and the GP needs to attend urgently where security cannot be arranged, the policy must be approached.
- (c) **Consultations on practice premises** – booked appointments for potentially violent patients should be made in advance so that the local security firm can be given notice of the appointment time (preferably at least 24 hours).

4 Informing Patients

The independent contractor should inform patients if a security guard is to act as an escort. The security guard will not be present during the consultation, but will remain in close proximity in order to provide support and security as required. The security guard will search the patient prior to any consultation.

5 How to access the service

GPs and their staff should telephone the security company, Apollo Security, on:

01562 827160

07774 201813

07710 246493

6 Payment arrangements

The local security firm is contracted on a cost per case basis. Practices should therefore confirm with the Primary Care Team by e-mail the attendance and time on site of the security guard. This will allow the PCT to audit the invoice received from the local security firm.

FAX TO REGISTRATION TEAM on 01905 760001

PATIENT REMOVAL FORM

for cases of VIOLENT or AGGRESSIVE BEHAVIOUR

Details of Patient

Name:

Address:

Date of Birth:

NHS number:

Currently registered GP/practice:

Date of registration at that practice:

Reason for Removal

		Physical	Verbal
Assault to:	Practitioner		
	Staff		
	Other patient		
	Racial/sexual harassment		
	Other (please state)		
Vandalism to:	Premises		
	Vehicle		

Details of Practitioner/Member of Staff - Time/Location of Incident

Name and designation:

Address of premises:

.....

Date of incident:

Time of incident:

Location of Incident:

Surgery/patient's home, etc:

Report to the Police

Name and number of the Officer:

Police station:

Incident number:

Details Relating to Removal Request

Summary of Incident

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Additional Information

Is this an isolated incident or have there been others?

.....

Are incidents gender specific (eg aggression/violence only towards females)?

.....

Are there complicating factors (drug/alcohol abuse, severe mental health problems, learning disabilities, language barriers) ?

.....

Are there any mitigating factors?

.....

Additional Comments

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Patient informed by practice of decision that they be removed ☐

Is the patient currently under the care of any other healthcare professional?

Awaiting new outpatient appointment with:

.....

Under outpatient follow up with:

.....

Receiving community care from:

.....

Other:

.....

Signature: Date:

Name (please print):

Position:

***NOTES TO BE RETURNED TO ISAAC MADDOX HOUSE
AS SOON AS POSSIBLE***

FOR PCT USE ONLY

Approved for Tier 2? yes ☐ no ☐

Authorised signature:

Name:

Date:

If approved

Please e-mail the Tier 2 Service with basic patient details **and**

Fax this form to Tier 2 Service on **01527 576022**

Inform any other healthcare professionals the patient is under the care of (as listed above)

Equality Impact Assessment – Stage 1 (Screening Template)

Policy or service being assessed	Policy and Procedure for the Management of Abusive, Difficult and Violent Patients in General Practice
Person(s) involved in conducting the assessment	Lynda Dando, Head of Primary Care Nigel Higenbottam, Deputy Head of Primary Care Jo Hall, Primary Care Contracts Manager
Date the assessment is completed	30 April 2010
Directorate Lead	Lynda Dando
Policy or service lead (Job title and contact details)	Nigel Higenbottam, Deputy Head of Primary Care

1. Is this a new or existing policy or service?	New ☐	Existing ☒				
What are the aims, objectives and purpose of the policy or service? (include how it fits into strategic objectives)						
The policy is designed to support general practitioners and their staff in dealing with abusive, violent and difficult patients.						
2. Who will benefit from the policy or service and how?						
	Give a brief explanation of how the target audience will benefit?					
Service Users	Reduces the risk of patients being the subject of or witness to verbal or physical abuse.					
Staff	Provides a safer working environment for general practitioners and their staff by dealing with patients who are verbally or physically abusive.					
Partner Organisations	Provides a mechanism for informing partner organisations of those patients for whom alternative arrangements are in place the provision of primary care services.					
3. Does the policy or service contain aspects which cause inequality? E.g. Location of facilities, operational hours etc.						
No						
4. For each of the six equality strands, answer the questions in the table below with 'yes' or 'no', with a brief explanation of your decision.						
Question	Race	Gender	Disability	Sexual Orientation	Age	Religion or Belief
a. Do different groups have different needs,	No	No	No	No	No	No

experiences, issues and priorities in relation to the proposed policy or service?						
b. Is there potential for or evidence that the proposed policy or service will not promote equality of opportunity for all and promote good relations between different groups?	No	No	No	No	No	No
c. Is there potential for or evidence that the proposed policy or service will affect different population groups differently (including possibly discriminating against certain groups)?	No	No	No	No	No	No
d. Is there public concern (including media, academic, voluntary or sector specific interest) in policy or service area about actual, perceived or potential discrimination against a particular population group or groups?	No	No	No	No	No	No
5. If you answer yes to any of the questions, what evidence supports this?						

Present your evidence stating the question followed by the strand(s) For example: Question: a. Strand: disability and sexual orientation.....followed by the evidence			
N/A			
6. Provider Services only to answer this question. Are there concerns that the policy or service could have an adverse (negative) impact on terms following human rights principles (FREDA): (See Guidance notes Appendix A)			
Fairness	Yes	☐	No ☐
Respect	Yes	☐	No ☐
Equality	Yes	☐	No ☐
Dignity	Yes	☐	No ☐
Autonomy	Yes	☐	No ☐
7. If you answer yes to any of the principles, what evidence supports this? Present your evidence as the principle followed by the evidence.			
N/A			
8. Have any barriers been identified that could inhibit access to the benefits of the policy or service e.g. physical access, information etc?			
No			
9. How will the service or policy be monitored in relation to the six strands and human rights?			
This policy will be monitored by examining specific cases of violence and aggression which have been reported by practices.			
10. What level of adverse (negative) impact (high or low) will this policy or service change have on each of the equality groups?			
Race	Low		
Gender	Low		
Disability	Low		
Sexual Orientation	Low		
Age	Low		
Religion or Belief	Low		
11. What action will you be taking to address these issues?			
Please complete the action plan.			
12. Should the policy or service proceed to a Full Equality Impact Assessment? If so, what are the reasons?			
No			

Equality Impact Assessment Report Template

Your Equality Impact Assessment Report should demonstrate what you do (or will do) to make sure that your function/policy is accessible to different people and communities, not just that it can, in theory, be used by anyone.

1. Name of policy or function – Policy and Procedure for the Management of Abusive, Difficult and Violent Patients in General Practice
2. Responsible Manager – Nigel Higenbottam, Deputy Head of Primary Care
3. Date EIA completed – 30 April 2010
4. Description of aims of function/policy – Provide guidance to GP practices on the reporting and future management of violent, abusive and difficult patients.
5. Brief summary of research and relevant data
6. Methods and outcomes of consultation
7. Results of Initial Screening or Full Equality Impact Assessment

Initial or Full Equality Impact Assessment?	
Equality Group	Assessment of Impact
Race	Nil
Gender	Nil
Disability	Nil
Age	Nil
Sexual Orientation	Nil
Religion or Belief	Nil
Human Rights	Nil

8. Decisions and or recommendations (including supporting rationale)
9. Equality action plan (if required) – N/A
10. Monitoring and review arrangements (include date of next full review)

Department	Primary Care
Directorate	Primary Care
Director	Dr Jonathan Leach
Report produced by and job title	Nigel Higenbottam, Deputy Head of Primary Care
Date report produced	30 April 2010
Date report published	